



Institute for The Study Of Humanity

A research initiative of Maximized Impact ry,
a Finnish registered association. Helsinki.

First look

Study 7: telephone anxiety and call avoidance

464 responses received September 28 to October 7, 2026 · Report dated October 9, 2026

*A preliminary result of an ongoing study. The figures describe
the people who answered, not the general population.*

This is the first look at the results of Study 7 - Stage A, an anonymous survey on how phone calls feel to adults with ADHD, autism, both (AuDHD) or neither. It covers every response received before 00:00 Helsinki time on October 8, 2026. That cutoff was fixed in the preregistration before any data were collected.

To our knowledge, no published study has measured how common telephone anxiety or call avoidance is among adults with ADHD or among autistic adults. The survey remains open, and the Wave 1 report follows after October 31, 2026.

Main figures

Measure	All 464 respondents
Telephone anxiety, broad definition Answered Often or Always to at least one of “When my phone rings, I feel tense or anxious” and “Before a call I need to make, I feel anxious,” or rated the anxiety that phone calls cause at 7 or more on a scale of 0 to 10. This is the headline measure.	71% 95% confidence interval 67–75
Telephone anxiety, strict definition Answered Often or Always to both statements.	51% 95% confidence interval 46–55
Call avoidance Answered Often or Always to at least one of “I let calls from people I know go unanswered because of how calls make me feel” and “I put off returning calls for a day or more,” or had in the past year avoided something important because it required a phone call.	80% 95% confidence interval 76–83

By group

Each respondent reported their own ADHD and autism status: diagnosed, suspected (in assessment, or not diagnosed but identifying or suspecting) or no. There is no control group. Every group is described in its own right.

Group	n	Telephone anxiety, broad	Telephone anxiety, strict	Call avoidance
All respondents	464	71% (67–75)	51% (46–55)	80% (76–83)
ADHD diagnosed, autism diagnosed (AuDHD)	48	98% (89–100)	77% (63–87)	98% (89–100)
ADHD diagnosed, autism suspected	88	83% (74–89)	67% (57–76)	90% (82–95)
ADHD diagnosed, no autism (ADHD-only)	30	63% (46–78)	40% (25–58)	77% (59–88)
ADHD suspected, autism diagnosed	13	85% (58–96)	54% (29–77)	92% (67–99)
ADHD suspected, autism suspected	100	76% (67–83)	58% (48–67)	83% (74–89)
ADHD suspected, no autism	50	48% (35–61)	22% (13–35)	66% (52–78)
No ADHD, autism diagnosed (autism-only)	21	90% (71–97)	81% (60–92)	81% (60–92)
No ADHD, autism suspected	35	86% (71–94)	63% (46–77)	91% (78–97)
No ADHD, no autism (neither)	79	41% (30–52)	15% (9–25)	56% (45–66)
<i>All with diagnosed ADHD</i>	166	84% (77–89)	65% (58–72)	90% (84–94)
<i>All with diagnosed autism</i>	82	94% (87–97)	74% (64–83)	93% (85–97)

n = respondents in the group. In parentheses: 95% confidence interval. The last two rows are totals of rows above.

Telephone anxiety by the broad definition was most common in the group with both ADHD and autism diagnosed (98%) and least common in the group with neither (41%).

How to read these figures

The sample is self-selected. People chose to answer a survey about phone calls, and it was shared in ADHD and autism communities and in general-audience channels. Those who find calls hard may have been more likely to answer. The percentages describe these respondents and do not show how common telephone anxiety is in the population. Differences between groups are more robust than any single percentage.

Who answered

Background	Respondents
Language version	Finnish 443 · English or Swedish 21
Region of residence	Finland 453 · outside Finland 11
Age	18–24: 31 · 25–34: 95 · 35–44: 178 · 45–54: 129 · 55 or older: 31
Gender	woman 302 · man 121 · non-binary 26 · prefer not to say 15
ADHD	diagnosed 166 · suspected 163 · no 135
Autism	diagnosed 82 · suspected 223 · no 159
Anxiety	diagnosed 175 · suspected 128 · no 161

What respondents answered

Share of all 464 respondents who answered Often or Always. The 95% confidence interval is under each figure.

When the phone rings

Question	Share of respondents
1. When my phone rings, I feel tense or anxious.	56% 52–61
2. Before a call I need to make, I feel anxious.	64% 59–68
3. Overall, how much anxiety do phone calls cause you? (0 = none, 10 = extreme)	60% 7 or more · 56–65

What you do about it

Question	Share of respondents
4. I let calls from people I know go unanswered because of how calls make me feel.	26% 22–30
5. I put off returning calls for a day or more.	39% 34–43
6. In the past year I have avoided something important (an appointment, a bill, a job, a health matter) because it required a phone call.	76% Yes · 72–80
7. I answer calls from unknown numbers.	23% 20–27

Time and ending

Question	Share of respondents
8. During calls I lose track of how long the call has lasted.	39% 35–44
9. I find it hard to end a call even when I want to.	33% 29–37

Not knowing what to expect

Question	Share of respondents
10. When I don't know why someone is calling, I feel anxious.	54% 49–58
11. When I don't know how long a call will last, I feel anxious.	32% 28–36

Preference

Question	Share of respondents
12. Given the choice, I prefer messaging to calling.	74% 70–78

How the figures were made

Preregistration

The definitions, the cutoff and the exclusion rules were registered on the Open Science Framework on September 28, 2026, before any data were collected: <https://osf.io/8t473>.

Responses included

All 464 responses received before the cutoff. The three registered data-quality rules removed none: there were no test submissions, no session code appeared twice, and no response had less than 45 seconds of active answering time (the shortest was 76 seconds). Every response is complete, because the survey cannot be sent with a required question unanswered. Three responses were sent when the bot check could not run, and one respondent reported having answered the survey before. As registered, these responses are included. Leaving out either set changes no main figure by as much as half a percentage point.

Confidence intervals and small groups

The 95% confidence intervals are Wilson intervals. The registration requires cells of fewer than ten respondents to be suppressed. Here that rule is applied as follows: no figure is shown for a group of fewer than ten respondents, and background categories with fewer than ten respondents are merged.

Not yet done

This first look reports shares with their confidence intervals only. The registered hypotheses (H1 to H5) are tested on the Wave 1 data. The registered target is at least 300 respondents in each of the four hypothesis groups (ADHD-only, autism-only, AuDHD and neither). They now have 21 to 79.

Ethics review

The survey is anonymous, stores no personal data, involves no intervention and is open only to adults. Under the guidelines of the Finnish National Board on Research Integrity (TENK), research of this kind does not require ethical pre-review, and none was sought.

Limits

- The sample is self-selected, so the figures are not population estimates.
- All information is self-reported. Diagnoses were not verified.
- 443 of the 464 respondents answered the Finnish version, so the results so far mostly describe people who answered in Finnish.
- Figures for small groups are imprecise. The autism-only group, for example, has 21 respondents.
- The questions other than the ASRS screener were translated with terminology verification but without back-translation, which limits comparability between language versions.
- The survey is a single cross-section and supports no conclusions about cause and effect.

What happens next

The survey remains open at research.maximized-impact.org/study7. Wave 1 closes at 23:59 Helsinki time on October 31, 2026. Its results are published as Addendum 1 to the white paper “The Case for Health Communications Technology” (<https://doi.org/10.5281/zenodo.21978705>), with a summary in Finnish.

The figures may change as more people answer. This first look is not used to decide when data collection stops.

Open data and code

The anonymized dataset behind this first look, and the code that recomputes every result in it from that dataset, are published with this report at research.maximized-impact.org/study7/results under the CC BY 4.0 license. In the dataset, age group, gender, region and language are withheld wherever fewer than ten respondents share a combination, so the background counts above cannot be recomputed from it. Submission times are not included.

Conflict of interest and funding

The author developed Yapper Phone, a commercial application. The survey’s optional tools (call timer, focus sounds and time signals) were adapted from it. The study measures how common a problem is. It makes no claim about the application and does not use it as an intervention. The Institute for The Study Of Humanity is funded in part by the application’s profits through Maximized Impact ry, a Finnish registered association that cannot be sold or converted into a for-profit entity. Funding: Maximized Impact ry. There is no external funding.

Citation and contact

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Appendix: every question by group

Share answering Often or Always, in percent (question 3: 7 or more; question 6: Yes). The 95% confidence interval is in parentheses. Q1 to Q12 refer to the numbered questions above.

Group	n	Q1	Q2	Q3	Q4	Q5	Q6
All respondents	464	56 (52–61)	64 (59–68)	60 (56–65)	26 (22–30)	39 (34–43)	76 (72–80)
ADHD diagnosed, autism diagnosed (AuDHD)	48	81 (68–90)	92 (80–97)	94 (83–98)	40 (27–54)	58 (44–71)	96 (86–99)
ADHD diagnosed, autism suspected	88	74 (64–82)	74 (64–82)	75 (65–83)	45 (35–56)	56 (45–66)	84 (75–90)
ADHD diagnosed, no autism (ADHD-only)	30	43 (27–61)	60 (42–75)	50 (33–67)	17 (7–34)	30 (17–48)	73 (56–86)
ADHD suspected, autism diagnosed	13	62 (36–82)	77 (50–92)	77 (50–92)	15 (4–42)	69 (42–87)	85 (58–96)
ADHD suspected, autism suspected	100	62 (52–71)	71 (61–79)	68 (58–76)	27 (19–36)	39 (30–49)	78 (69–85)
ADHD suspected, no autism	50	30 (19–44)	38 (26–52)	30 (19–44)	18 (10–31)	24 (14–37)	62 (48–74)
No ADHD, autism diagnosed (autism-only)	21	81 (60–92)	90 (71–97)	81 (60–92)	33 (17–55)	43 (24–63)	81 (60–92)
No ADHD, autism suspected	35	66 (49–79)	83 (67–92)	74 (58–86)	23 (12–39)	49 (33–64)	89 (74–95)
No ADHD, no autism (neither)	79	24 (16–35)	27 (18–37)	23 (15–33)	4 (1–11)	9 (4–17)	53 (42–64)
<i>All with diagnosed ADHD</i>	166	70 (63–77)	77 (70–82)	76 (69–82)	39 (31–46)	52 (44–59)	86 (79–90)
<i>All with diagnosed autism</i>	82	78 (68–86)	89 (80–94)	88 (79–93)	34 (25–45)	56 (45–66)	90 (82–95)

Group	n	Q7	Q8	Q9	Q10	Q11	Q12
All respondents	464	23 (20–27)	39 (35–44)	33 (29–37)	54 (49–58)	32 (28–36)	74 (70–78)
ADHD diagnosed, autism diagnosed (AuDHD)	48	19 (10–32)	56 (42–69)	67 (53–78)	83 (70–91)	54 (40–67)	88 (75–94)
ADHD diagnosed, autism suspected	88	22 (14–31)	50 (40–60)	42 (32–52)	76 (66–84)	50 (40–60)	78 (69–86)
ADHD diagnosed, no autism (ADHD-only)	30	23 (12–41)	47 (30–64)	23 (12–41)	37 (22–54)	10 (3–26)	57 (39–73)
ADHD suspected, autism diagnosed	13	31 (13–58)	54 (29–77)	46 (23–71)	77 (50–92)	54 (29–77)	100 (77–100)
ADHD suspected, autism suspected	100	24 (17–33)	37 (28–47)	36 (27–46)	56 (46–65)	34 (25–44)	80 (71–87)
ADHD suspected, no autism	50	34 (22–48)	26 (16–40)	12 (6–24)	28 (17–42)	12 (6–24)	64 (50–76)
No ADHD, autism diagnosed (autism-only)	21	0 (0–15)	43 (24–63)	52 (32–72)	86 (65–95)	62 (41–79)	95 (77–99)
No ADHD, autism suspected	35	14 (6–29)	26 (14–42)	37 (23–54)	57 (41–72)	29 (16–45)	89 (74–95)
No ADHD, no autism (neither)	79	29 (20–40)	27 (18–37)	6 (3–14)	18 (11–28)	6 (3–14)	51 (40–61)
<i>All with diagnosed ADHD</i>	166	21 (16–28)	51 (44–59)	46 (38–53)	71 (64–77)	44 (37–52)	77 (70–83)
<i>All with diagnosed autism</i>	82	16 (10–25)	52 (42–63)	60 (49–70)	83 (73–90)	56 (45–66)	91 (83–96)